



89%.

undiagnosed.
untreated.
incarcerated.

Of children assessed in an Australian youth detention centre, 89% had at least one severe neurodevelopmental impairment. Most had never been diagnosed. Three quarters were Aboriginal.

we did assess them. after we locked them up.

89%

had at least one severe neurodevelopmental impairment

36%

had fetal alcohol spectrum disorder

65%

were impaired across three or more domains

74%

of those assessed were Aboriginal

Bower et al., BMJ Open 2018 — Banksia Hill Detention Centre, Western Australia. 99 young people aged 10–17 received a full assessment. The highest prevalence of fetal alcohol spectrum disorder recorded in any youth custodial centre in the world.

the justice targets are going backwards.

 4 / 19 Closing the Gap targets on track, halfway to the 2031 deadline	 3% → 37% share of the adult population, against share of people in custody	 2,500 Aboriginal and Torres Strait Islander adults imprisoned per 100,000 — up from 1,925 in 2019	 No change Target 11, youth detention, against a 30% reduction by 2031
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Productivity Commission Closing the Gap dashboard and Annual Data Compilation (July 2026); ABS Prisoners in Australia (2025) and Corrective Services (March quarter 2026). Target 10 seeks a 15% reduction in adult imprisonment by 2031; Target 11 a 30% reduction in youth detention.

one in four people in custody.

ADHD is not evenly distributed across the population. It is concentrated, at extraordinary rates, in exactly the group the justice targets are failing.

25.5%

ADHD prevalence across incarcerated populations,
on diagnostic interview

30.1%

in youth custody — around five times the general
population

26.2%

in adult custody — around ten times the general
population

Young et al., meta-analysis of ADHD prevalence in incarcerated populations; diagnostic-interview estimates. These are international figures. No Australian Aboriginal and Torres Strait Islander-specific prevalence study exists — which is itself a finding, and one this pilot would begin to address.

32% fewer convictions. the same people.

A Swedish national cohort of 25,656 people with ADHD, followed from 2006 to 2009. Criminality fell 32% in men and 41% in women during periods on medication.

The comparison is within-individual.

Not treated people against untreated people, where the two groups differ in a hundred ways. The same person, on medication and off it. That design is why the finding survives.

Lichtenstein et al., "Medication for Attention-Deficit-Hyperactivity Disorder and Criminality", New England Journal of Medicine, 22 November 2012. Swedish national registers; stratified Cox regression.

ADHD does not cause incarceration.

WHAT DRIVES IT

Colonisation and dispossession, poverty, housing, over-policing, bail laws, and the age of criminal responsibility. Nothing in this proposal touches any of them.

WHAT IS ALSO TRUE

Among children already in contact with the system, severe neurodevelopmental impairment is close to universal, and mostly undiagnosed until a court asks.

WHAT WE PROPOSE

Treating a treatable condition in people who are not currently offered treatment. That is health care, not a theory of crime.

A health intervention cannot fix a justice problem whose causes are structural. What it can do is stop the health system contributing to one, by treating a condition it currently leaves undiagnosed until a court is involved.

prevention starts years earlier.



01

Identified at school age

Not at sentencing. The MBS 715 health assessment already exists as the annual contact point, and is already funded.



02

Assessed close to home

By a GP who has done the training — which Queensland and New South Wales now permit, and other states are following.



03

Treated and reviewed

Follow-up on a schedule with one clinician, rather than a single appointment and no return.



04

Counted

Against a randomised holdout, so the effect is measured rather than assumed.

one year of one cell, or nine hundred assessments.

\$3,600	\$1.3m	~900	\$159,510
one child, one day, in youth detention	one child, one year	ADHD assessments the same year would fund	one adult, one year, in prison

Detention is the most expensive thing this system does, and the least effective. Assessment is among the cheapest.

Report on Government Services 2026 (youth justice and corrective services). Assessment cost taken at the private-market rate cited in this proposal; delivered through a community-controlled service the per-assessment cost is lower, and the ratio improves.

a register, a recall, and a receipt.

Two years of domain-neutral primary-care infrastructure, running end to end against a synthetic practice engine. It identifies who is eligible, invites them to their own clinician, and measures attendance against a randomised holdout.

- ✓ Identify — a register built from the service's own records, inside rules it sets.
- ✓ Invite — a plain, availability-only message; the patient books with their own clinician.
- ✓ Count — attended assessments only, against a randomised holdout.

Incrementality

26 simulated weeks · 4,000 synthetic patients

INCREMENTAL
ATTENDED / 1,000

120.6

North star — invite-
arm rate above
holdout

INCREMENTAL
ATTENDED

392

vs naive generated
count 582 (contrast
only)

INVITE ARM

1430

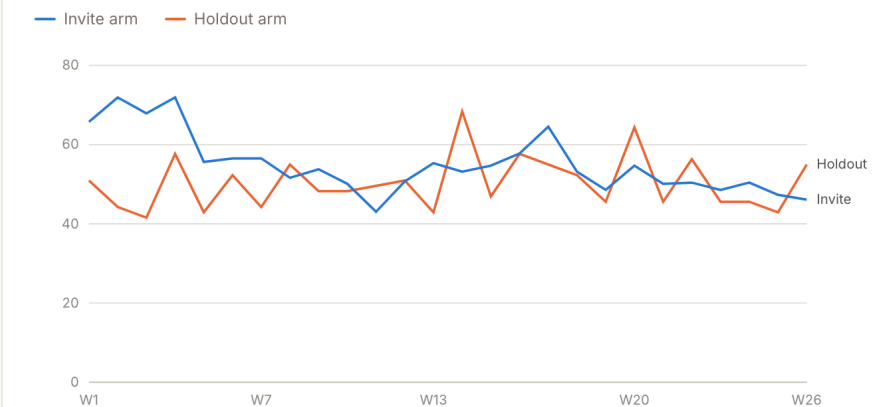
attended / 1,000 ·
3,254 patients

HOLDOUT ARM

1310

attended / 1,000 · 746
patients

Weekly attended per 1,000 — by arm



Attended appointments per 1,000 arm patients per week. Both arms share the same organic process; the gap is the generated lift.

Weekly table

opt-outs 1.2% of 2,952 sent

Synthetic practice engine — 26 simulated weeks, 4,000 synthetic patients. No real patient data exists in this build, and none will without the partner organisation's decision.

WHOSE INFRASTRUCTURE THIS IS

built to be handed over.

PRIORITY REFORM TWO

Deployed as the partner organisation’s own infrastructure. We never hold the patient relationship, the clinical record, or the Medicare claim.

PRIORITY REFORM FOUR

The partner organisation is the data custodian. Nothing is disclosed, published or shared without its decision.

PRIORITY REFORM ONE

The partner sets eligibility, message content, and whether the pilot proceeds at all — written into the agreement rather than left to goodwill.

NO PARTNER YET

We have no Aboriginal or Torres Strait Islander partner organisation today. Securing one is Phase 0, before any build.

one region, twelve months, one auditable number.

PHASE 0 · MONTHS 1–3

Partnership first. No build, no data and no deployment until an Aboriginal Community Controlled Health Organisation has agreed scope, governance and data terms in writing.

PHASE 1 · MONTHS 4–9

Identification and assessment pathway inside partner services, under the partner’s rules, with the randomised holdout running from the first day.

PHASE 2 · MONTHS 10–12

Read-out: assessments completed, treatment initiated, treatment retained at three months — and a plain account of what did not work.

PUBLICATION

Published whether positive or null, with the partner as co-author, and the underlying data remaining theirs.

Twelve months cannot measure incarceration, and this proposal does not pretend otherwise. The pilot measures the health steps the evidence links to it — assessment completed, treatment initiated, treatment retained — and reports those. Deliberately unpriced until co-designed in Phase 0.

three of us. the fourth seat is not ours
to fill.



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Stefan Thottunkal

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NOURISH, Stanford Medicine · Health
Systems Innovation Lab, Harvard T.H. Chan



Vacant, deliberately

ABORIGINAL AND TORRES STRAIT ISLANDER
GOVERNANCE

A condition of this work proceeding — not
an advisory seat added after funding.

four things, in this order.

-  An introduction to NACCHO or a state affiliate, so that partnership begins with the sector rather than with us.
-  Scoped pilot funding, released in two tranches — partnership first, build second.
-  Access to linked health and justice data at regional level, on Priority Reform Four terms set by the partner.
-  A named contact in Health and one in the Attorney-General's portfolio, because this sits across both.

If the holdout shows no effect, that is the finding, and we will publish it.

thank you

Vikram Ganeshalingam

Dr Anubhav Saxena

Stefan Thottunkal

We acknowledge Aboriginal and Torres Strait Islander peoples as the Traditional Owners and Custodians of this country, and pay respect to Elders past and present. Any work described here proceeds only with, and under the authority of, the communities it concerns.

add contact email before sending